GSSSB · STAFF NURSE EXAM

Mental Health Nursing MCQ Bank

430 MCQs · GSSSB pattern · Answer key with explanation



12 Sections | 430 MCQs | Direct + Priority + Scenario | Match & Assertion-Reason | Calculations | Explanations

Hinglish explanation + English exam terms



Sister Asha
Aapki Nursing teacher

MCQ Bank – Sections

1. **Introduction, History, Terms & Defence Mechanisms**
   30 MCQs · Pioneers, classification, psychiatric signs & symptoms, Freud's structure, defence mechanisms, neurotransmitters
2. **Assessment, Communication & Nurse–Patient Relationship**
   30 MCQs · MSE, rating scales, therapeutic & non-therapeutic communication, Johari window, transference, Peplau's phases, anxiety levels
3. **Psychopharmacology**
   30 MCQs · Antipsychotics & EPS/ NMS, antidepressants, serotonin syndrome, lithium, mood stabilisers, anxiolytics, other psychotropics, nursing responsibilities
4. **ECT & Therapeutic Modalities**
   30 MCQs · ECT indications, contraindications, pre/ post care, side effects; CBT, behaviour therapy, psychoanalysis, group, family, milieu, crisis intervention, restraint
5. **Schizophrenia & Psychotic Disorders**
   30 MCQs · Aetiology, 4 A's, first-rank symptoms, positive/ negative symptoms, types, prognosis, treatment, nursing care, other psychoses
6. **Mood Disorders & Suicide**
   30 MCQs · Depression, mania, bipolar, dysthymia, cyclothymia, treatment & nursing care, suicide risk factors, warning signs, precautions
7. **Anxiety, OCD, Stress, Dissociative & Somatoform Disorders**
   30 MCQs · Anxiety levels, GAD, panic, phobias, OCD, PTSD, adjustment, grief, conversion, dissociative, somatoform, factitious
8. **Substance Use Disorders**
   30 MCQs · Terminology, alcohol – screening, withdrawal, DT, Wernicke-Korsakoff, treatment; opioids, cannabis, stimulants, tobacco; nursing, laws
9. **Personality, Eating, Sleep & Sexual Disorders**
   30 MCQs · PD clusters, borderline & antisocial PD nursing, anorexia & bulimia, sleep disorders & hygiene, sexual dysfunctions & paraphilias
10. **Neurocognitive & Child-Adolescent Disorders**
    30 MCQs · Delirium vs dementia, Alzheimer's, nursing care, intellectual disability, autism, ADHD, conduct disorder, tic disorders, geriatric psychiatry
11. **Emergencies, Community Mental Health & MHCA 2017**
    30 MCQs · Violence management, psychiatric emergencies, community psychiatry, NMHP/ DMHP, rehabilitation, MHCA 2017 sections, forensic psychiatry, ethics
12. **GSSSB Pattern Mock Test – 100 MCQs (Full Mental Health Syllabus)**
    100 MCQs · 100 questions, 100 marks, 0.25 negative marking – complete Mental Health (Psychiatric) Nursing

Level 1 – Direct: facts, values, definitions

Level 2 – Priority: "first / best / immediate" action

Level 3 – Advanced: scenario, NOT/EXCEPT, match, A-R, calculation



MCQ kaise solve karein?

Pehle khud answer socho, phir options padho. Har section ke baad answer key aur explanation hai, galti wale questions ka number apni notebook me likho. Exam me galat answer par **0.25 marks** kat sakte hain, isliye bina soche guess mat karo.

1 Introduction, History, Terms & Defence Mechanisms

Level 1 – Direct

1. Mental patients ki chains hatane wale –
   (a) Kraepelin (b) Philippe Pinel (c) Bleuler (d) Freud
2. "Schizophrenia" term kisne diya?
   (a) Bleuler (b) Schneider (c) Freud (d) Kraepelin
3. Mother of psychiatric nursing –
   (a) Florence Nightingale (b) Linda Richards (c) Hildegard Peplau (d) Dorothea Dix
4. ECT ki shuruaat –
   (a) Cade (b) Cerletti & Bini (c) Sakel (d) Moniz
5. Lithium mania ke liye kisne introduce kiya?
   (a) John Cade (b) Delay (c) Kuhn (d) Beck
6. ICD kaun publish karta hai?
   (a) WHO (b) APA (c) UNICEF (d) ICMR
7. Bina external stimulus ke perception –
   (a) Illusion (b) Hallucination (c) Delusion (d) Obsession
8. Real stimulus ka galat perception –
   (a) Delusion (b) Hallucination (c) Illusion (d) Phobia
9. Fixed false belief –
   (a) Illusion (b) Hallucination (c) Delusion (d) Obsession
10. Sabse common delusion –
    (a) Persecution (b) Jealousy (c) Grandeur (d) Nihilistic
11. Flight of ideas kis me?
    (a) Mania (b) Schizophrenia (c) Dementia (d) Depression
12. Confabulation –
    (a) Phobia (b) OCD (c) Mania (d) Korsakoff psychosis
13. Waxy flexibility –
    (a) Mania (b) Catatonia (c) Delirium (d) Anxiety
14. Id kis principle par chalta hai?
    (a) Logic (b) Morality (c) Pleasure (d) Reality
15. Conscious defence mechanism –
    (a) Projection (b) Suppression (c) Repression (d) Denial
16. Sabse mature defence mechanism –
    (a) Denial (b) Sublimation (c) Regression (d) Projection
17. Disorientation me sabse pehle kya lose hota hai?
    (a) Place (b) Person (c) Time (d) Self

18. Alzheimer's me kaunsa neurotransmitter kam?
(a) GABA (b) Dopamine (c) Acetylcholine (d) Serotonin

? Level 2 – Priority & Application

19. Alcoholic kehta hai "mujhe peene ki koi problem nahi". Defence mechanism –
(a) Denial (b) Undoing (c) Sublimation (d) Projection

20. Boss se daant khane ke baad pita ghar par bachche par chillata hai –
(a) Rationalisation (b) Displacement (c) Regression (d) Compensation

21. Exam me fail student kehta hai "paper syllabus ke bahar tha" –
(a) Identification (b) Repression (c) Rationalisation (d) Denial

22. Gussa wale ladke ko boxing me daalna – energy acceptable outlet me –
(a) Sublimation (b) Displacement (c) Undoing (d) Projection

23. Hospital me 8 saal ka bachcha phir se bistar geela karne laga –
(a) Regression (b) Conversion (c) Fixation (d) Denial

24. Rope ko saanp samajhna –
(a) Depersonalisation (b) Illusion (c) Delusion (d) Hallucination

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. Pinel Q. Freud R. Peplau S. Kraepelin — with 1. Dementia praecox 2. Interpersonal theory 3. Psychoanalysis 4. Moral treatment
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-3, R-2, S-1 (c) P-4, Q-2, R-3, S-1 (d) P-3, Q-4, R-2, S-1

26. Match: P. Clang association Q. Neologism R. Perseveration S. Echolalia — with 1. Catatonia 2. Organic brain disease 3. Schizophrenia 4. Mania
(a) P-4, Q-3, R-2, S-1 (b) P-1, Q-3, R-2, S-4 (c) P-4, Q-2, R-3, S-1 (d) P-3, Q-4, R-2, S-1

27. Assertion (A): Mood subjective hota hai aur affect objective. Reason (R): Mood patient batata hai, affect nurse observe karti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Psychosis ka feature NAHI –
(a) Full insight present (b) Hallucinations (c) Delusions (d) Loss of contact with reality

29. Dopamine excess kis se associated?
(a) Myasthenia (b) Alzheimer's (c) Schizophrenia (d) Parkinsonism

30. Defence mechanism ka galat example –
(a) Undoing – guilt cancel karna (b) Reaction formation – ulta vyavhar (c) Projection – apni bhavna dusre par (d) Suppression – unconscious bhoolna

✓ Answer Key & Explanation

1-b Pinel 1793.
2-a Bleuler.
3-c Peplau.
4-b Cerletti & Bini 1938.
5-a John Cade 1949.
6-a WHO.
7-b Hallucination.
8-c Illusion.
9-c Delusion.
10-a Persecution.
11-a Mania.
12-d Korsakoff.

13-b Catatonia.
14-c Pleasure.
15-b Suppression.
16-b Sublimation.
17-c Time.
18-c ACh.
19-a Denial.
20-b Displacement.
21-c Rationalisation.
22-a Sublimation.
23-a Regression.
24-b Illusion.
25-b Sahi.
26-a Sahi.
27-a Dono sahi, R reason hai.
28-a Psychosis me insight poor.
29-c Schizophrenia.
30-d Suppression conscious hota hai.

2 Assessment, Communication & Nurse–Patient Relationship

Level 1 – Direct

1. MSE me abstract thinking kaise check karte hain?
   (a) Orientation (b) Digit span (c) Serial 7s (d) Proverb interpretation
2. Attention/ concentration test –
   (a) Clock drawing (b) Serial 7s subtraction (c) Insight (d) Proverbs
3. MMSE ka maximum score –
   (a) 10 (b) 20 (c) 30 (d) 50
4. Suicide risk scale –
   (a) AIMS (b) CAGE (c) Y-BOCS (d) SAD PERSONS
5. Alcohol screening –
   (a) MMSE (b) HAM-D (c) CAGE (d) PANSS
6. Tardive dyskinesia scale –
   (a) GAD-7 (b) PHQ-9 (c) BPRS (d) AIMS
7. Rorschach inkblot test –
   (a) IQ test (b) Memory test (c) Projective personality test (d) Depression scale
8. Johari window kis se related?
   (a) Pain (b) Memory (c) Intelligence (d) Self-awareness
9. Patient nurse ke prati apni maa jaisi feelings dikhata hai –
   (a) Projection (b) Empathy (c) Counter-transference (d) Transference
10. Nurse ki patient ke prati emotional reaction –
    (a) Counter-transference (b) Transference (c) Rapport (d) Sympathy
11. Peplau ke pehle phase me –
    (a) Discharge (b) Trust, contract, introduction (c) Working on problems (d) Termination
12. Therapeutic relationship ki key –
    (a) Empathy (b) Sympathy (c) Pity (d) Friendship
13. "Aap kis baare me baat karna chahenge?" – technique –
    (a) Advice (b) Broad opening (c) Closed question (d) Reassurance
14. Non-therapeutic response –
    (a) Silence (b) "Don't worry, sab theek ho jayega" (c) Reflection (d) Active listening
15. "Why" questions kyun avoid?
    (a) Doctor ka kaam (b) Time waste (c) Patient defensive ho jata hai (d) Rule only
16. Test judgement ka example –
    (a) "Aaj kaunsa din hai?" (b) "100 − 7" (c) "Apna naam?" (d) "Sadak par stamped letter mile to kya karoge?"
17. Panic level anxiety me nurse –
    (a) Akela chhod de (b) Group activity (c) Lecture de (d) Patient ke saath rahe, short simple directions

18. Insight ki grading –
(a) 1–6 (b) 1–10 (c) 1–3 (d) 0–30

? Level 2 – Priority & Application

19. Patient: "Mere kaan me awaazein aati hain". Best response –
(a) "Wo awaazein kya keh rahi hain?" (assess) (b) "Ignore karo" (c) "Awaazein nahi hoti" (d) Hasna

20. Patient rote hue kehta hai "meri wife mujhe chhod gayi". Therapeutic response –
(a) "Ro mat" (b) "Kyun chhodi?" (c) "Aap bahut dukhi lag rahe hain" (reflection) (d) "Doosri mil jayegi"

21. Patient chup baitha hai. Nurse –
(a) Chali jaaye (b) Daante (c) Therapeutic silence – saath baithe (d) Turant topic badle

22. Patient nurse ko gift dena chahta hai. Best –
(a) Professional boundary samjhakar politely mana karna (b) Gussa (c) Badle me gift (d) Le lena

23. Termination phase me patient udaas hai. Nurse –
(a) Phone number de de (b) Feelings express karne de, progress review kare (c) Ignore (d) Relationship continue rakhne ka vada

24. Moderate anxiety wale patient ko –
(a) Isolation (b) Feelings identify karne aur coping me madad (c) Complex tasks (d) Sedation always

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. Orientation Q. Identification R. Exploitation S. Resolution — with 1. Termination 2. Full use of services 3. Patient relates to nurse 4. Trust & contract
(a) P-3, Q-4, R-2, S-1 (b) P-4, Q-3, R-2, S-1 (c) P-1, Q-3, R-2, S-4 (d) P-4, Q-2, R-3, S-1

26. Therapeutic technique NAHI –
(a) Summarising (b) Restating (c) Giving advice (d) Clarification

27. Assertion (A): Termination ki baat orientation phase me hi shuru karni chahiye. Reason (R): Isse patient ka separation anxiety kam hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. MSE ka component NAHI –
(a) Mood & affect (b) Appearance (c) Thought & perception (d) Blood pressure

29. Johari window ke quadrants me NAHI –
(a) Hidden (b) Closed-loop (c) Blind (d) Open

30. SOLER me "L" –
(a) Leave (b) Look away (c) Lean towards patient (d) Listen loudly

✓ Answer Key & Explanation

1-d Proverbs.
2-b Serial 7s.
3-c 30.
4-d SAD PERSONS.
5-c CAGE.
6-d AIMS.
7-c Projective test.
8-d Self-awareness.
9-d Transference.
10-a Counter-transference.
11-b Orientation.
12-a Empathy.
13-b Broad opening.
14-b False reassurance.

15-c Defensive.
16-d Test judgement.
17-d Stay, simple directions.
18-a 1–6.
19-a Assess content (command?).
20-c Reflection.
21-c Silence.
22-a Boundary.
23-b Feelings & review.
24-b Coping.
25-b Sahi.
26-c Advice – non-therapeutic.
27-a Dono sahi, R reason hai.
28-d BP MSE ka part nahi.
29-b Unknown hota hai.
30-c Lean forward.

3 Psychopharmacology

Level 1 – Direct

1. Pehli antipsychotic –
   (a) Risperidone (b) Chlorpromazine (c) Clozapine (d) Haloperidol
2. Antipsychotics ka main mechanism –
   (a) Dopamine D2 blockade (b) ACh increase (c) Serotonin reuptake inhibition (d) GABA increase
3. Agranulocytosis ka risk –
   (a) Haloperidol (b) Risperidone (c) Aripiprazole (d) Clozapine
4. Hyperprolactinaemia (galactorrhoea) –
   (a) Quetiapine (b) Aripiprazole (c) Clozapine (d) Risperidone
5. Irreversible extrapyramidal side effect –
   (a) Tardive dyskinesia (b) Acute dystonia (c) Parkinsonism (d) Akathisia
6. Acute dystonia ka treatment –
   (a) Haloperidol more (b) IM promethazine / benztropine (c) Lithium (d) Propranolol
7. Akathisia ka treatment –
   (a) Digoxin (b) Propranolol (c) Clozapine (d) Insulin
8. NMS ka drug –
   (a) Dantrolene (b) Haloperidol (c) Lithium (d) Fluoxetine
9. Depot antipsychotic –
   (a) Fluoxetine (b) Lithium (c) Fluphenazine decanoate (d) Diazepam
10. First-line antidepressant group –
    (a) MAOI (b) SSRI (c) BZD (d) TCA
11. Sabse lambi half-life wala SSRI –
    (a) Escitalopram (b) Sertraline (c) Paroxetine (d) Fluoxetine
12. Overdose me sabse dangerous (cardiotoxic) –
    (a) Buspirone (b) Mirtazapine (c) TCA (d) SSRI
13. Cheese reaction –
    (a) Clozapine (b) MAOI (c) Lithium (d) SSRI
14. Lithium therapeutic level –
    (a) 0.1–0.4 (b) 0.6–1.2 mEq/L (c) 2–3 (d) 5–10
15. Lithium ka teratogenic effect –
    (a) Phocomelia (b) Cleft lip (c) Ebstein's anomaly (d) NTD
16. BZD overdose antidote –
    (a) Atropine (b) Naloxone (c) Physostigmine (d) Flumazenil
17. Non-addictive anxiolytic –
    (a) Alprazolam (b) Lorazepam (c) Diazepam (d) Buspirone
18. Alzheimer's ki dawa –
    (a) Donepezil (b) Haloperidol (c) Methylphenidate (d) Lithium

Level 2 – Priority & Application

19. Haloperidol lene ke kuch ghante baad patient ki gardan ek taraf mud gayi, aankhen upar. Kya hai?
(a) Tardive dyskinesia (b) Acute dystonia (c) Seizure (d) NMS

20. Antipsychotic par patient ko tez bukhar, rigidity, confusion, ↑ CPK. Nurse ka pehla kaam –
(a) Antipsychotic band karke doctor ko inform (NMS) (b) Walk karana (c) Paracetamol (d) Dose badhana

21. Lithium patient ko diarrhoea, vomiting, coarse tremor. Nurse –
(a) Next dose hold, serum lithium level, doctor inform (b) Dose double (c) Ignore (d) Salt restrict

22. Lithium patient ko diet advice –
(a) Normal salt aur 2–3 L fluids (b) Salt bilkul band (c) Diuretics lo (d) Fluid restriction

23. MAOI patient ko kya nahi khana?
(a) Rice (b) Aged cheese, wine, pickled fish (c) Fresh vegetables (d) Water

24. Clozapine patient ka regular kaunsa test?
(a) ECG only (b) Urine sugar (c) X-ray (d) WBC / ANC count

25. Antidepressant start karne ke 2 hafte baad patient ki energy badhi par mood low hai. Nurse kya dhyan rakhe?
(a) Kuch nahi (b) Dawa band (c) Suicide risk badha – close observation (d) Discharge

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Match: P. Acute dystonia Q. Akathisia R. Parkinsonism S. Tardive dyskinesia — with 1. Months–years, irreversible 2. Weeks 3. Days–weeks restlessness 4. Hours–days
(a) P-4, Q-3, R-2, S-1 (b) P-3, Q-4, R-2, S-1 (c) P-1, Q-3, R-2, S-4 (d) P-4, Q-2, R-3, S-1

27. Lithium level badhane wali dawa NAHI –
(a) Thiazide diuretic (b) Normal saline infusion (c) ACE inhibitor (d) NSAIDs

28. Assertion (A): Lithium patient ko dehydration se bachana chahiye. Reason (R): Sodium aur fluid kam hone par kidney lithium zyada reabsorb karti hai aur toxicity hoti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Anticholinergic side effect NAHI –
(a) Excess salivation (b) Constipation (c) Dry mouth (d) Urinary retention

30. Serotonin syndrome ka antidote –
(a) Cyproheptadine (b) Naloxone (c) Flumazenil (d) Dantrolene

Answer Key & Explanation

1-b Chlorpromazine 1952.
2-a D2 blockade.
3-d Clozapine.
4-d Risperidone.
5-a TD.
6-b Anticholinergic/ antihistamine IM.
7-b Propranolol.
8-a Dantrolene.
9-c Fluphenazine decanoate.
10-b SSRI.
11-d Fluoxetine.
12-c TCA.
13-b MAOI + tyramine.
14-b 0.6–1.2.
15-c Ebstein's.
16-d Flumazenil.
17-d Buspirone.
18-a Donepezil.
19-b Acute dystonia.
20-a NMS – stop drug.
21-a Toxicity.
22-a Normal salt, fluids.
23-b Tyramine foods.
24-d WBC/ ANC.

25-c Suicide risk.
26-a Sahi.
27-b NS lithium excretion badhata hai.
28-a Dono sahi, R reason hai.
29-a Anticholinergic se dry mouth (clozapine exception).
30-a Cyproheptadine.

4 ECT & Therapeutic Modalities

Level 1 – Direct

1. ECT ka sabse common indication –
   (a) Severe depression with suicidal risk (b) Personality disorder (c) Insomnia (d) Anxiety
2. ECT ka most important relative contraindication –
   (a) Pregnancy (b) Raised intracranial pressure (c) Old age (d) Diabetes
3. ECT se pehle NPO –
   (a) 24 hrs (b) Not needed (c) 1–2 hrs (d) 6–8 hrs
4. ECT me muscle relaxant –
   (a) Thiopentone (b) Atropine (c) Diazepam (d) Succinylcholine
5. ECT premedication for secretions –
   (a) Morphine (b) Insulin (c) Adrenaline (d) Atropine / glycopyrrolate
6. ECT ka sabse common side effect –
   (a) Death (b) Blindness (c) Memory loss (temporary) (d) Fracture
7. ECT course –
   (a) 6–12 sessions, 2–3/week (b) Daily 30 sessions (c) Monthly (d) 1 session
8. CBT kisne develop kiya?
   (a) Freud (b) Moreno (c) Aaron Beck (d) Skinner
9. Phobia ke liye systematic desensitisation –
   (a) Joseph Wolpe (b) Pavlov (c) Rogers (d) Ellis
10. OCD ki psychotherapy of choice –
    (a) Aversion (b) Hypnosis (c) Exposure & response prevention (d) Psychoanalysis
11. Alcoholism me aversion therapy ka principle –
    (a) Free association (b) Insight (c) Reward (d) Unpleasant stimulus ko behaviour ke saath pair karna
12. Token economy –
    (a) ECT (b) Psychoanalysis (c) Operant conditioning – rewards (d) Classical conditioning
13. Free association aur dream analysis –
    (a) Milieu (b) Psychoanalysis (c) Group therapy (d) CBT
14. Therapeutic community –
    (a) Beck (b) Caplan (c) Peplau (d) Maxwell Jones
15. Crisis intervention kitne samay me resolve?
    (a) 1 year (b) 4–6 weeks (c) 6 months (d) 24 hrs
16. Psychodrama –
    (a) Perls (b) Moreno (c) Frankl (d) Rogers
17. Seasonal affective disorder ka treatment –
    (a) Surgery (b) Light therapy (c) Lithium only (d) ECT only
18. Client-centred therapy –
    (a) Skinner (b) Wolpe (c) Carl Rogers (d) Freud

Level 2 – Priority & Application

19. ECT se pehle patient ke liye kya NAHI karna?
(a) Dentures nikalna (b) Consent lena (c) Baalon me tel lagana (d) Bladder empty

20. ECT ke baad patient confused hai. Nurse ka pehla kaam –
(a) Airway, lateral position, vitals, re-orientation (b) Discharge (c) Walk (d) Turant khana

21. ECT ke baad oral fluids kab?
(a) Kabhi nahi (b) Gag reflex aur full consciousness ke baad (c) Turant (d) 24 hrs baad

22. Aggressive patient ke liye restraint kab?
(a) Raat bhar (b) De-escalation aur medication fail hone par last resort (c) Sabse pehle (d) Punishment ke liye

23. Restrained patient ka circulation check –
(a) Har 15 min (b) Har 4 hrs (c) Shift end (d) Kabhi nahi

24. Schizophrenia patient ki family ko kis cheez ki training se relapse kam?
(a) Patient ko ghar me band karna (b) Criticism (c) Dawa band (d) Expressed emotion kam karna (psychoeducation)

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. CBT Q. Systematic desensitisation R. ERP S. Aversion therapy — with 1. Alcoholism 2. OCD 3. Phobia 4. Depression
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-3, R-2, S-1 (c) P-4, Q-2, R-3, S-1 (d) P-3, Q-4, R-2, S-1

26. ECT indication NAHI –
(a) Catatonia (b) Treatment-resistant mania (c) Suicidal depression (d) Antisocial personality disorder

27. Assertion (A): ECT me succinylcholine dete hain. Reason (R): Ye muscle relax karke seizure se fracture/ injury rokta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Yalom ke group therapeutic factors me NAHI –
(a) Catharsis (b) Punishment (c) Instillation of hope (d) Universality

29. Beck ka cognitive triad –
(a) Negative view of self, world, future (b) Mood, affect, thought (c) Alarm, resistance, exhaustion (d) Id, ego, superego

30. MHCA 2017 ke anusar ECT –
(a) Sirf anaesthesia & muscle relaxant ke saath; minors me prohibited (b) Banned completely (c) Bachchon me routine (d) Kabhi bhi bina anaesthesia

Answer Key & Explanation

1-a Severe depression.
2-b Raised ICP.
3-d 6–8 hrs.
4-d Succinylcholine.
5-d Atropine.
6-c Memory loss.
7-a 6–12.
8-c Beck.
9-a Wolpe.
10-c ERP.
11-d Pairing unpleasant stimulus.
12-c Operant.
13-b Psychoanalysis.
14-d Maxwell Jones.
15-b 4–6 weeks.
16-b Moreno.
17-b Light therapy.
18-c Rogers.

19-c Hair oil-free hona chahiye.
20-a Post-ECT care.
21-b Gag reflex return.
22-b Last resort.
23-a 15 min.
24-d Reduce EE.
25-b Sahi.
26-d Not indication.
27-a Dono sahi, R reason hai.
28-b Not a factor.
29-a Cognitive triad.
30-a Sec 95.

5 Schizophrenia & Psychotic Disorders

Level 1 – Direct

1. Schizophrenia ka lifetime prevalence –
   (a) 0.1 % (b) 1 % (c) 10 % (d) 20 %
2. Schizophrenia ka neurotransmitter hypothesis –
   (a) ACh deficiency (b) Dopamine excess (c) GABA excess (d) Serotonin deficiency
3. Bleuler ke 4 A's me NAHI –
   (a) Affect (b) Amnesia (c) Autism (d) Ambivalence
4. First-rank symptoms kisne diye?
   (a) Kraepelin (b) Bleuler (c) Kurt Schneider (d) Freud
5. First-rank symptom –
   (a) Thought broadcasting (b) Flight of ideas (c) Compulsion (d) Early morning awakening
6. Schizophrenia ka sabse common type –
   (a) Hebephrenic (b) Paranoid (c) Catatonic (d) Simple
7. Worst prognosis wala type –
   (a) Paranoid (b) Hebephrenic (disorganised) (c) Undifferentiated (d) Catatonic
8. Negative symptom –
   (a) Hallucination (b) Alogia / avolition (c) Delusion (d) Bizarre behaviour
9. DSM-5 me schizophrenia diagnosis ke liye duration –
   (a) 6 months (b) 1 month (c) 1 week (d) 2 years
10. Treatment-resistant schizophrenia –
    (a) Lithium (b) Clozapine (c) Haloperidol (d) Fluoxetine
11. Catatonic schizophrenia ka treatment –
    (a) Lorazepam / ECT (b) Methylphenidate (c) Disulfiram (d) SSRI
12. Shared psychotic disorder –
    (a) Capgras (b) Folie à deux (c) Cotard (d) Othello
13. "Mera sharir mar chuka hai" – delusion –
    (a) Capgras (b) Erotomania (c) Fregoli (d) Cotard (nihilistic)
14. Familiar person ko impostor maanna –
    (a) Dhat (b) Cotard (c) Capgras syndrome (d) Koro
15. Morbid jealousy –
    (a) Munchausen (b) Ganser (c) Othello syndrome (d) de Clérambault
16. Postpartum psychosis –
    (a) Normal blues (b) Anxiety only (c) Personality disorder (d) Psychiatric emergency (infanticide risk)
17. Schizophrenia me hallucination usually –
    (a) Visual (b) Auditory (c) Olfactory (d) Tactile
18. Dhat syndrome –
    (a) Penis retraction fear (b) Semen loss anxiety (c) Spirit possession (d) Eating disorder

Level 2 – Priority & Application

19. Patient kehta hai "TV mujhse baat kar raha hai". Best response –
(a) "Pagal ho kya" (b) "Haan TV bolta hai" (c) Ignore (d) "Main samajh sakti hu aapko aisa lagta hai, par TV kisi se baat nahi karta"

20. Patient khana nahi khata, kehta hai zeher mila hai. Nurse –
(a) Sealed/ packaged food de ya patient khud plate chune (b) Khana band (c) Bahas kare (d) Force feed

21. Patient ko command hallucination aa rahi hai "khud ko maar do". Priority –
(a) Lecture (b) Discharge (c) Ignore (d) Safety – close observation, doctor ko inform

22. Withdrawn schizophrenia patient ke liye pehla step –
(a) One-to-one brief, frequent interactions (b) Competitive game (c) Turant bada group (d) Akela chhodna

23. Catatonic stupor patient ki nursing priority –
(a) Debate (b) Group therapy (c) Nutrition, fluids, skin care, position change (d) Exercise class

24. Paranoid patient ke saath –
(a) Honest, consistent, saamne whisper na karna (b) Achanak touch (c) Secrets (d) Mazaak

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Good prognostic factor NAHI –
(a) Late onset (b) Good premorbid adjustment (c) Prominent negative symptoms (d) Acute onset

26. Match: P. Paranoid Q. Hebephrenic R. Catatonic S. Simple — with 1. Insidious negative symptoms only 2. Waxy flexibility, stupor 3. Silly affect, disorganised 4. Persecutory delusions
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-2, R-3, S-1 (c) P-4, Q-3, R-2, S-1 (d) P-3, Q-4, R-2, S-1

27. Assertion (A): Delusion par patient se bahas nahi karni chahiye. Reason (R): Delusion fixed belief hai, bahas se patient aur defensive ho jata hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Negative symptoms me NAHI –
(a) Thought insertion (b) Anhedonia (c) Affective flattening (d) Asociality

29. High expressed emotion ka matlab –
(a) Therapy (b) Family me criticism, hostility, over-involvement – relapse ↑ (c) Medicine (d) Happy family

30. Monozygotic twin me schizophrenia concordance approx –
(a) 12 % (b) 1 % (c) ~48 % (d) 100 %

Answer Key & Explanation

1-b 1 %.
2-b Dopamine excess.
3-b Amnesia nahi – association hai.
4-c Schneider.
5-a Thought broadcasting.
6-b Paranoid.
7-b Hebephrenic.
8-b Alogia/ avolition.
9-a 6 months.
10-b Clozapine.
11-a Lorazepam/ ECT.
12-b Folie à deux.
13-d Cotard.
14-c Capgras.
15-c Othello.
16-d Emergency.
17-b Auditory.
18-b Semen loss.
19-d Present reality, acknowledge feelings.
20-a Sealed food.
21-d Safety.
22-a One-to-one.
23-c Physical needs.
24-a Honesty, consistency.

25-c Negative symptoms – poor.
26-c Sahi.
27-a Dono sahi, R reason hai.
28-a Thought insertion – positive (FRS).
29-b High EE.
30-c ~48 %.

6 Mood Disorders & Suicide

Level 1 – Direct

1. Depression diagnosis ke liye minimum duration –
   (a) 1 month (b) 2 weeks (c) 3 days (d) 6 months
2. Depression ki typical sleep problem –
   (a) Night terrors (b) Early morning awakening (c) Sleepwalking (d) Hypersomnia always
3. Depression me diurnal variation –
   (a) Subah zyada kharab (b) Koi variation nahi (c) Raat ko best (d) Shaam ko kharab
4. Mania ka minimum duration –
   (a) 1 day (b) 1 week (c) 2 weeks (d) 4 days
5. Hypomania + major depression –
   (a) Bipolar I (b) Dysthymia (c) Cyclothymia (d) Bipolar II
6. 2 saal tak mild depression –
   (a) Bipolar I (b) Dysthymia (c) Cyclothymia (d) Mania
7. Bipolar disorder ki prophylaxis ka gold standard –
   (a) Haloperidol (b) Diazepam (c) Lithium (d) Fluoxetine
8. Learned helplessness theory –
   (a) Beck (b) Freud (c) Selye (d) Seligman
9. Suicide ka strongest predictor –
   (a) Previous suicide attempt (b) Young age only (c) Female sex (d) Married
10. India me suicide ka sabse common method –
    (a) Poisoning (b) Firearms (c) Hanging (d) Drowning
11. Completed suicide zyada –
    (a) Equal (b) Females (c) Children (d) Males
12. Suicide risk assessment scale –
    (a) AIMS (b) SAD PERSONS (c) CIWA (d) Y-BOCS
13. World Suicide Prevention Day –
    (a) 10 September (b) 10 October (c) 26 June (d) 7 April
14. Suicide attempt decriminalised –
    (a) RPwD (b) NDPS (c) MHA 1987 (d) MHCA 2017 Sec 115
15. Mania me speech –
    (a) Mutism (b) Poverty of speech (c) Normal (d) Pressure of speech
16. Seasonal affective disorder –
    (a) Personality (b) Winter depression – light therapy (c) Child disorder (d) Summer mania only
17. Psychotic depression me delusions usually –
    (a) None (b) Grandeur (c) Guilt, poverty, nihilism (mood-congruent) (d) Erotomania
18. Suicide most associated psychiatric illness –
    (a) Depression (b) Dementia (c) OCD (d) Phobia

Level 2 – Priority & Application

19. Depressed patient achanak shant aur khush lagne laga, saman baant raha hai. Nurse –
(a) Ignore (b) Celebrate (c) Discharge (d) Suicide risk high – close observation, assess plan

20. Patient se suicide ke baare me poochhna –
(a) Risk nahi badhata – directly poochhna chahiye (b) Kabhi na poochhe (c) Sirf doctor poochhe (d) Risk badhata hai

21. Suicidal patient ke room me kya nahi rakhna?
(a) Bed (b) Water jug (plastic) (c) Pillow (d) Belts, shoelaces, sharp objects, cords

22. Manic patient ke liye activity –
(a) Card games competition (b) Walking / sweeping (non-competitive gross motor) (c) Chess (d) Football match

23. Manic patient khana nahi khata, bhaag-daud karta hai. Diet –
(a) Formal 3-course meal (b) Liquid only (c) Fasting (d) High-calorie finger foods & fluids

24. Depressed patient ke saath nurse ka approach –
(a) "Cheer up" kehna (b) Simple achievable tasks, time spend, accept silence (c) Ignore (d) Complex decisions

25. Manic patient ke liye environment –
(a) Quiet, low-stimulus single room (b) TV room (c) Loud music (d) Crowded dayroom

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. DIG FAST (mania) me NAHI –
(a) Grandiosity (b) Hypersomnia (c) Distractibility (d) Flight of ideas

27. Match: P. Bipolar I Q. Bipolar II R. Cyclothymia S. Dysthymia — with 1. Mild depression ≥ 2 yrs 2. Mild mood swings ≥ 2 yrs 3. Hypomania + depression 4. Full mania
(a) P-4, Q-3, R-2, S-1 (b) P-4, Q-2, R-3, S-1 (c) P-3, Q-4, R-2, S-1 (d) P-1, Q-3, R-2, S-4

28. Assertion (A): Antidepressant ke shuru ke hafton me suicide risk badh sakta hai. Reason (R): Energy pehle lautti hai, mood baad me sudharta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. SAD PERSONS me "P" –
(a) Previous attempt (b) Parents (c) Poverty (d) Pain

30. Suicide warning sign NAHI –
(a) Possessions de dena (b) Future plans with enthusiasm & social engagement (c) Will banana (d) Hopelessness

Answer Key & Explanation

1-b 2 weeks.
2-b Early morning awakening.
3-a Morning worse.
4-b 1 week.
5-d Bipolar II.
6-b Dysthymia.
7-c Lithium.
8-d Seligman.
9-a Previous attempt.
10-c Hanging.
11-d Males.
12-b SAD PERSONS.
13-a 10 Sept.
14-d Sec 115.
15-d Pressure.
16-b Light therapy.
17-c Mood-congruent.
18-a Depression.

19-d Sudden calm – high risk.
20-a Asking is safe.
21-d Harmful objects.
22-b Gross motor.
23-d Finger foods.
24-b Simple tasks.
25-a Low stimulus.
26-b ↓ sleep need hota hai.
27-a Sahi.
28-a Dono sahi, R reason hai.
29-a Previous attempt.
30-b Protective, not warning.

7 Anxiety, OCD, Stress, Dissociative & Somatoform Disorders

Level 1 – Direct

1. GAD duration –
   (a) 1 month (b) 2 years (c) 6 months (d) 2 weeks
2. Heights ka dar –
   (a) Nyctophobia (b) Claustrophobia (c) Acrophobia (d) Agoraphobia
3. Band jagah ka dar –
   (a) Acrophobia (b) Xenophobia (c) Zoophobia (d) Claustrophobia
4. Bheed / open place ka dar –
   (a) Thanatophobia (b) Social phobia (c) Hydrophobia (d) Agoraphobia
5. Andhere ka dar –
   (a) Pyrophobia (b) Mysophobia (c) Nyctophobia (d) Algophobia
6. Phobia ka treatment –
   (a) Systematic desensitisation (b) ECT (c) Psychosurgery (d) Lithium
7. OCD me sabse common compulsion –
   (a) Praying (b) Counting (c) Hoarding (d) Hand washing
8. OCD ka drug –
   (a) Disulfiram (b) Lithium (c) Haloperidol (d) SSRI / clomipramine
9. PTSD symptoms kitne samay se zyada?
   (a) 2 days (b) 1 year (c) 1 month (d) 1 week
10. PTSD ka feature –
    (a) Confabulation (b) Flashbacks, avoidance, hyperarousal (c) Grandiosity (d) Waxy flexibility
11. General Adaptation Syndrome –
    (a) Hans Selye (b) Caplan (c) Beck (d) Freud
12. Kübler-Ross grief stages ka pehla stage –
    (a) Bargaining (b) Anger (c) Denial (d) Acceptance
13. La belle indifférence –
    (a) OCD (b) Conversion disorder (c) Depression (d) Mania
14. Achanak ghar chhod kar door jaana + amnesia –
    (a) Delirium (b) Schizophrenia (c) Dissociative fugue (d) Amnesia only
15. Gambhir bimari hone ka lagatar dar despite normal tests –
    (a) Malingering (b) Phobia (c) Hypochondriasis (illness anxiety) (d) Conversion
16. External gain ke liye jaan boojh kar bimari ka natak –
    (a) Factitious disorder (b) Somatisation (c) Malingering (d) Conversion
17. Sick role ke liye jaan boojh kar symptoms –
    (a) Munchausen (factitious) (b) Hypochondriasis (c) Conversion (d) Malingering

18. Anxiety me kaunsa neurotransmitter kam?
(a) Glutamate excess only (b) GABA (c) Dopamine (d) Acetylcholine

? Level 2 – Priority & Application

19. Panic attack ke dauran nurse –
(a) Lecture (b) Akela chhode (c) Restraint (d) Patient ke saath rahe, shant awaaz, slow breathing

20. OCD patient hand washing ritual kar raha hai. Nurse –
(a) Daantna (b) Paani band (c) Turant rokna (d) Shuru me ritual ke liye time de, dhire dhire limit

21. Conversion paralysis wale patient ke saath –
(a) Ignore completely (b) Mazaak (c) Bar-bar test (d) Symptoms par focus kam, stressors par baat, secondary gain na de

22. Hypochondriac patient ke liye –
(a) Feelings par focus, unnecessary investigations avoid (b) Surgery (c) Kehna "kuch nahi hai" (d) Har din naye test

23. Disaster ke 2 mahine baad survivor ko nightmares aur flashbacks. Suspect –
(a) Mania (b) Acute stress reaction (c) PTSD (d) Schizophrenia

24. Pseudoseizure ka pehchaan –
(a) Normal prolactin, no post-ictal confusion (b) Abnormal EEG (c) ↑ prolactin (d) Tongue bite, incontinence

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. Acrophobia Q. Hydrophobia R. Xenophobia S. Thanatophobia — with 1. Death 2. Strangers 3. Water 4. Heights
(a) P-4, Q-3, R-2, S-1 (b) P-3, Q-4, R-2, S-1 (c) P-4, Q-2, R-3, S-1 (d) P-1, Q-3, R-2, S-4

26. OCD ka feature NAHI –
(a) Insight usually present (b) Thoughts apne lagte hain (c) Thoughts bahar se daale gaye lagte hain (d) Patient resist karta hai

27. Assertion (A): OCD patient ka ritual achanak rokne se anxiety badhti hai. Reason (R): Compulsion anxiety kam karne ke liye kiya jata hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Holmes & Rahe scale me sabse zyada stress score –
(a) Retirement (b) Death of spouse (c) Marriage (d) Divorce

29. Panic level anxiety ka feature NAHI –
(a) Disorganised thinking (b) Loss of control (c) Terror (d) Enhanced learning

30. GAS ke stages –
(a) Orientation, working, termination (b) Alarm, resistance, exhaustion (c) Denial, anger, bargaining (d) Mild, moderate, severe

✓ Answer Key & Explanation

1-c 6 months.
2-c Acrophobia.
3-d Claustrophobia.
4-d Agoraphobia.
5-c Nyctophobia.
6-a Desensitisation.
7-d Washing.
8-d SSRI.

9-c > 1 month.
10-b PTSD triad.
11-a Selye.
12-c Denial.
13-b Conversion.
14-c Fugue.
15-c Hypochondriasis.
16-c Malingering.
17-a Factitious.
18-b GABA.
19-d Stay with patient.
20-d Gradual limit.
21-d Avoid secondary gain.
22-a Feelings focus.
23-c PTSD.
24-a Pseudoseizure.
25-a Sahi.
26-c That's thought insertion (schizophrenia).
27-a Dono sahi, R reason hai.
28-b Death of spouse (100).
29-d Learning impaired.
30-b GAS.

8 Substance Use Disorders

Level 1 – Direct

1. Same effect ke liye zyada dose ki zarurat –
   (a) Craving (b) Relapse (c) Withdrawal (d) Tolerance
2. CAGE questionnaire kis ke liye?
   (a) Alcohol (b) Anxiety (c) Depression (d) Dementia
3. Alcohol ka sabse sensitive lab marker –
   (a) Creatinine (b) GGT (c) Sugar (d) Hb
4. Alcohol withdrawal ka earliest sign –
   (a) Tremors (b) DT (c) Seizures (d) Hallucinosis
5. Delirium tremens kab?
   (a) 2 weeks (b) 6 hrs (c) 12 hrs (d) 48–72 hrs
6. DT me hallucinations –
   (a) Olfactory only (b) None (c) Visual (Lilliputian) (d) Gustatory
7. Alcohol withdrawal me drug of choice –
   (a) Disulfiram (b) Benzodiazepine (chlordiazepoxide) (c) Haloperidol (d) Naloxone
8. Liver disease me alcohol detox ke liye BZD –
   (a) Chlordiazepoxide (b) Lorazepam (c) Diazepam (d) Clonazepam
9. Wernicke's encephalopathy kis deficiency se?
   (a) Thiamine (b) Niacin (c) B12 (d) Folate
10. Korsakoff ka characteristic –
    (a) Hallucination (b) Mania (c) Seizure (d) Amnesia + confabulation
11. Disulfiram kis enzyme ko inhibit karta hai?
    (a) Aldehyde dehydrogenase (b) Alcohol dehydrogenase (c) COMT (d) MAO
12. Alcohol craving kam karne wali dawa –
    (a) Naltrexone / acamprosate (b) Disulfiram only (c) Haloperidol (d) Lithium
13. Opioid overdose antidote –
    (a) Flumazenil (b) Naloxone (c) Atropine (d) Physostigmine
14. Opioid intoxication me pupils –
    (a) Unequal (b) Dilated (c) Pin-point (d) Normal
15. Opioid substitution therapy –
    (a) Buprenorphine / methadone (b) Haloperidol (c) Disulfiram (d) Diazepam
16. Cannabis ka long-term effect –
    (a) Korsakoff (b) Amotivational syndrome (c) Pellagra (d) Wernicke
17. NDPS Act –
    (a) 1971 (b) 1985 (c) 2005 (d) 2017
18. Alcoholics Anonymous –
    (a) Drug (b) 12-step self-help programme (c) Government scheme (d) Hospital

Level 2 – Priority & Application

19. Alcohol dependent patient ko IV glucose dena hai. Pehle kya?
(a) IV thiamine (b) Insulin (c) Disulfiram (d) Haloperidol

20. Admission ke 3rd din patient confused, kaanp raha, deewar par keede dekh raha, bukhar. Suspect –
(a) Schizophrenia (b) OCD (c) Delirium tremens (d) Mania

21. DT patient ka room –
(a) Akela band (b) Well-lit, quiet, re-orientation (c) Noisy ward (d) Dark room

22. Disulfiram dene se pehle –
(a) Alcohol ke saath (b) Kabhi bhi (c) Patient ki informed consent, 12–24 hrs abstinence (d) Chupke se khaane me

23. Disulfiram patient ko kya avoid karna?
(a) Fruits (b) Rice (c) Alcohol wale cough syrup, mouthwash (d) Water

24. Addiction me sabse common defence mechanism –
(a) Altruism (b) Humour (c) Denial (d) Sublimation

25. Heroin user achanak band karta hai – yawning, aansu, naak behna, dilated pupils. Ye –
(a) Cocaine (b) Opioid withdrawal (c) Alcohol intoxication (d) Cannabis

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Match: P. Tremors Q. Rum fits R. Hallucinosis S. DT — with 1. 48–72 hrs 2. Auditory hallucination clear consciousness 3. 12–48 hrs seizures 4. 6–12 hrs
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-2, R-3, S-1 (c) P-4, Q-3, R-2, S-1 (d) P-3, Q-4, R-2, S-1

27. Wernicke's triad me NAHI –
(a) Ataxia (b) Confusion (c) Confabulation (d) Ophthalmoplegia

28. Assertion (A): Alcohol withdrawal me benzodiazepines di jaati hain. Reason (R): Alcohol aur BZD me cross-tolerance hota hai (GABA).
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Opioid withdrawal ka sign NAHI –
(a) Pin-point pupils (b) Diarrhoea (c) Piloerection (d) Lacrimation

30. CAGE me "E" –
(a) Empathy (b) Emotion (c) Exercise (d) Eye-opener

Answer Key & Explanation

1-d Tolerance.
2-a Alcohol.
3-b GGT.
4-a Tremors.
5-d 48–72 hrs.
6-c Visual.
7-b BZD.
8-b Lorazepam.
9-a Thiamine.
10-d Confabulation.
11-a ALDH.
12-a Naltrexone/ acamprosate.
13-b Naloxone.
14-c Pin-point.
15-a Buprenorphine/ methadone.
16-b Amotivational.
17-b 1985.
18-b 12-step.
19-a Thiamine first.
20-c DT.

21-b Well-lit.
22-c Consent + abstinence.
23-c Hidden alcohol.
24-c Denial.
25-b Opioid withdrawal.
26-c Sahi.
27-c Confabulation – Korsakoff.
28-a Dono sahi, R reason hai.
29-a Withdrawal me dilated.
30-d Eye-opener.

9 Personality, Eating, Sleep & Sexual Disorders

Level 1 – Direct

1. Cluster A personality disorders –
   (a) Odd/ eccentric (b) Psychotic (c) Anxious (d) Dramatic
2. Paranoid PD kis cluster me?
   (a) B (b) C (c) None (d) A
3. Lack of guilt, criminal behaviour –
   (a) Antisocial PD (b) Dependent PD (c) Schizoid PD (d) Avoidant PD
4. Self-harm, fear of abandonment, splitting –
   (a) Narcissistic (b) Schizotypal (c) Borderline PD (d) Histrionic
5. Borderline PD ki therapy –
   (a) ECT (b) Light therapy (c) Psychosurgery (d) DBT (Dialectical Behaviour Therapy)
6. Attention-seeking, dramatic –
   (a) Avoidant (b) Histrionic PD (c) Paranoid (d) Schizoid
7. Grandiose, lack of empathy –
   (a) Avoidant (b) Narcissistic PD (c) Schizotypal (d) Dependent
8. Cluster C me NAHI –
   (a) Avoidant (b) Obsessive-compulsive (c) Dependent (d) Antisocial
9. Psychiatric disorders me sabse zyada mortality –
   (a) GAD (b) Anorexia nervosa (c) Phobia (d) OCD
10. Anorexia nervosa ka BMI (ICD-10) –
    (a) < 17.5 (b) 20–25 (c) < 25 (d) > 30
11. Russell's sign (knuckle calluses) –
    (a) Pica (b) Bulimia nervosa (c) Obesity (d) Anorexia restricting
12. Refeeding syndrome me –
    (a) Hypophosphataemia (b) Hyperkalaemia (c) Hypercalcaemia (d) Hyperglycaemia only
13. Sabse common sleep disorder –
    (a) Sleep apnoea (b) Narcolepsy (c) Sleepwalking (d) Insomnia
14. Cataplexy –
    (a) Insomnia (b) Bruxism (c) Night terror (d) Narcolepsy
15. Narcolepsy ki dawa –
    (a) Diazepam (b) Lithium (c) Modafinil (d) Haloperidol
16. Sleepwalking kis stage me?
    (a) REM (b) Awake (c) Stage 1 (d) NREM stage 3 (deep)
17. Dusron ko kapde badalte chhupkar dekhna –
    (a) Exhibitionism (b) Fetishism (c) Voyeurism (d) Frotteurism

18. Painful intercourse –
(a) Anorgasmia (b) Dyspareunia (c) Vaginismus (d) Premature ejaculation

Level 2 – Priority & Application

19. Borderline patient ek nurse ki taarif aur doosri ki burai karta hai. Best approach –
(a) Patient ki baat maan lo (b) Favour wali nurse hi dekhe (c) Ignore (d) Sabhi staff consistent rules follow karein

20. Antisocial patient rules todta hai. Nurse –
(a) Special favour (b) Argue (c) Kuch nahi (d) Firm consistent limits & consequences

21. Anorexia patient ka weight –
(a) Patient batayega (b) Weekly guess (c) Kabhi bhi kapdon ke saath (d) Roz same time, same scale, gown me, void ke baad

22. Bulimia/ anorexia patient ko khane ke baad –
(a) 1–2 hrs supervise (vomiting rokne) (b) Turant bathroom (c) Akela (d) Exercise

23. Insomnia patient ko advice –
(a) Bed par TV (b) Regular schedule, no daytime naps, no caffeine at night (c) Raat ko coffee (d) Daytime long naps

24. Manipulative patient ke saath –
(a) Limits set & enforce consistently, power struggle avoid (b) Har maang poori (c) Gift lena (d) Secrets rakhna

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. Cluster A Q. Cluster B R. Cluster C S. OCPD — with 1. Perfectionism, rigidity 2. Anxious/ fearful 3. Dramatic/ erratic 4. Odd/ eccentric
(a) P-4, Q-3, R-2, S-1 (b) P-3, Q-4, R-2, S-1 (c) P-1, Q-3, R-2, S-4 (d) P-4, Q-2, R-3, S-1

26. Anorexia ka physical sign NAHI –
(a) Lanugo (b) Bradycardia (c) Hypertension & tachycardia (d) Hypothermia

27. Assertion (A): Borderline PD me staff splitting rokne ke liye consistent approach zaroori hai.
Reason (R): Patient staff ko "good" aur "bad" me baant kar manipulate karta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Sleep hygiene ka part NAHI –
(a) Avoid caffeine evening (b) Regular wake time (c) Daytime naps 3 hrs (d) Bed only for sleep

29. Bulimia ka feature NAHI –
(a) Self-induced vomiting (b) Binge eating (c) Dental erosion (d) Very low BMI always

30. Night terrors aur nightmares – sahi kya?
(a) Dono NREM (b) Night terror NREM, yaad nahi; nightmare REM, yaad rehta (c) Night terror REM (d) Dono REM

Answer Key & Explanation

1-a Odd.
2-d A.
3-a Antisocial.
4-c Borderline.
5-d DBT.
6-b Histrionic.

7-b Narcissistic.
8-d Antisocial – B.
9-b Anorexia.
10-a < 17.5.
11-b Bulimia.
12-a ↓ Phosphate.
13-d Insomnia.
14-d Narcolepsy.
15-c Modafinil.
16-d NREM 3.
17-c Voyeurism.
18-b Dyspareunia.
19-d Consistent approach.
20-d Firm limits.
21-d Standard weighing.
22-a Supervise.
23-b Sleep hygiene.
24-a Limits.
25-a Sahi.
26-c Hypotension, bradycardia hoti hai.
27-a Dono sahi, R reason hai.
28-c Naps avoid.
29-d Bulimia me weight usually normal.
30-b Sahi.

10 Neurocognitive & Child-Adolescent Disorders

Level 1 – Direct

1. Acute onset, fluctuating, attention impaired –
   (a) Dementia (b) Delirium (c) Schizophrenia (d) Depression
2. Dementia me consciousness –
   (a) Clear (b) Clouded (c) Comatose (d) Fluctuating always
3. Sabse common dementia –
   (a) Alzheimer's (b) Lewy body (c) Frontotemporal (d) Vascular
4. Alzheimer's me pehle kya prabhavit?
   (a) Vision (b) Remote memory (c) Motor power (d) Recent memory
5. Alzheimer's pathology –
   (a) Amyloid plaques & neurofibrillary tangles (b) Infarcts only (c) Tumour (d) Demyelination
6. Cholinesterase inhibitor –
   (a) Donepezil (b) Haloperidol (c) Memantine (d) Lithium
7. NPH triad me NAHI –
   (a) Hallucinations (b) Dementia (c) Urinary incontinence (d) Gait disturbance
8. Depression jo dementia jaisa dikhe –
   (a) Amnesia (b) Delirium (c) Pseudodementia (d) Fugue
9. Sundowning –
   (a) Sleepwalking (b) Shaam/ raat ko confusion badhna (c) Sun allergy (d) Subah confusion
10. Purani yaadon par based therapy –
    (a) ECT (b) Reminiscence therapy (c) Aversion (d) Reality orientation only
11. Intellectual disability me IQ –
    (a) < 50 only (b) < 70 (c) < 90 (d) < 100
12. Mild ID IQ range –
    (a) 20–34 (b) 35–49 (c) < 20 (d) 50–69
13. ID ka sabse common preventable cause –
    (a) Congenital hypothyroidism (b) Down syndrome (c) Autism (d) Fragile X
14. Autism kis age se pehle?
    (a) 3 years (b) 7 years (c) 12 years (d) 18 years
15. ADHD ki dawa –
    (a) Methylphenidate (b) Haloperidol (c) Lithium (d) Diazepam
16. Motor + vocal tics, coprolalia –
    (a) ADHD (b) Conduct disorder (c) Tourette's (d) Autism
17. Sabse common learning disorder –
    (a) Dyslexia (b) Dyscalculia (c) Dysgraphia (d) Dyspraxia

18. Elderly me sabse common psychiatric disorder –
(a) Mania (b) Depression (c) Schizophrenia (d) OCD

? Level 2 – Priority & Application

19. Post-op elderly patient raat ko confused, deewar par keede dekhta hai. Nursing –
(a) Restraint turant (b) Well-lit room, re-orientation, family presence, cause dhoondhna (c) Sedation heavy (d) Dark room

20. Dementia patient baar baar ghar se bahar nikal jata hai. Priority –
(a) Restraint lifelong (b) Safety – ID bracelet, locked exits, supervision (c) Ignore (d) Lecture

21. Dementia patient se baat karne ka tarika –
(a) Complex instructions (b) Argue (c) Loud scolding (d) Short simple one-step instructions, calm

22. Autistic child ke liye –
(a) No therapy (b) Frequent changes (c) Structured routine, consistent caregiver (d) Crowded play

23. ADHD child ke classroom ke liye –
(a) Front seat, short tasks, one instruction at a time (b) Back seat (c) Group noise (d) Long lectures

24. Delirium me kaunsa drug avoid (alcohol withdrawal ke alawa)?
(a) IV fluids (b) Thiamine (c) Benzodiazepines (d) Low-dose haloperidol

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Delirium vs dementia – delirium ka feature NAHI –
(a) Acute onset (b) Clear consciousness (c) Usually reversible (d) Fluctuating

26. Match: P. Donepezil Q. Memantine R. Methylphenidate S. Haloperidol (low dose) — with 1. Delirium agitation 2. ADHD 3. NMDA antagonist 4. Cholinesterase inhibitor
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-2, R-3, S-1 (c) P-3, Q-4, R-2, S-1 (d) P-4, Q-3, R-2, S-1

27. Assertion (A): Delirium me underlying cause ka treatment sabse important hai. Reason (R): Delirium aksar medical illness/ drugs ki wajah se hota hai aur reversible hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Mental age 8, chronological age 10. IQ –
(a) 60 (b) 80 (c) 100 (d) 125

29. Alzheimer's ke 4 A's me NAHI –
(a) Amnesia (b) Aphasia (c) Ambivalence (d) Apraxia

30. Reversible dementia cause NAHI –
(a) Alzheimer's disease (b) B12 deficiency (c) Hypothyroidism (d) Normal pressure hydrocephalus

✓ Answer Key & Explanation

1-b Delirium.
2-a Clear.
3-a Alzheimer's.
4-d Recent memory.
5-a Plaques & tangles.
6-a Donepezil.
7-a Hallucinations not in triad.
8-c Pseudodementia.
9-b Evening.
10-b Reminiscence.
11-b < 70.
12-d 50–69.
13-a Hypothyroidism.
14-a 3 yrs.

15-a Methylphenidate.
16-c Tourette's.
17-a Dyslexia.
18-b Depression.
19-b Delirium care.
20-b Safety.
21-d Simple instructions.
22-c Structure.
23-a Front seat.
24-c BZD worsen delirium.
25-b Delirium – clouded.
26-d Sahi.
27-a Dono sahi, R reason hai.
28-b 8/10 × 100 = 80.
29-c Agnosia hota hai.
30-a Alzheimer's irreversible.

11 Emergencies, Community Mental Health & MHCA 2017

Level 1 – Direct

1. Violence ka best predictor –
   (a) Old age (b) Past history of violence (c) Female sex (d) Education
2. Aggressive patient ke saath pehla step –
   (a) Injection (b) Isolation (c) Verbal de-escalation (d) Restraint
3. NMHP –
   (a) 1982 (b) 1987 (c) 1996 (d) 2017
4. DMHP pilot district –
   (a) Pune (b) Ranchi (c) Ahmedabad (d) Bellary
5. MHCA 2017 kab lagu hua?
   (a) 1 Jan 2018 (b) 7 April 2017 (c) 10 Oct 2017 (d) 29 May 2018
6. MHCA me advance directive –
   (a) Section 5 (b) Section 94 (c) Section 89 (d) Section 115
7. Emergency treatment MHCA me kitne ghante tak?
   (a) 7 days (b) 24 hrs (c) 30 days (d) 72 hrs
8. Supported admission (Sec 89) max –
   (a) 1 year (b) 7 days (c) 30 days (d) 90 days
9. MHCA me prohibited –
   (a) ECT without anaesthesia, ECT in minors, chaining (b) Antipsychotics (c) Admission (d) Counselling
10. District-level body (MHCA) –
    (a) MCI (b) INC (c) Mental Health Review Board (d) NHA
11. Tele-MANAS number –
    (a) 108 (b) 112 (c) 1098 (d) 14416
12. Half-way home –
    (a) ICU (b) OPD (c) Jail (d) Hospital aur community ke beech transitional facility
13. Insanity defence rule –
    (a) Tarasoff (b) Bolam (c) Durham (d) McNaughten rule
14. Will banane ki capacity –
    (a) Testamentary capacity (b) Criminal responsibility (c) Judgement (d) Insight
15. Third party ko warn karne ka duty –
    (a) McNaughten (b) Navtej (c) Tarasoff case (d) Selvi
16. Catatonic stupor ka emergency treatment –
    (a) Lithium (b) Disulfiram (c) Lorazepam / ECT (d) Fluoxetine
17. TCA overdose me cardiac toxicity ke liye –
    (a) Atropine (b) Flumazenil (c) Sodium bicarbonate (d) Naloxone

18. MHCA 2017 ke anusar seclusion –
(a) Prohibited (b) Allowed for punishment (c) Night only (d) Routine

? Level 2 – Priority & Application

19. Aggressive patient ke paas jaate waqt nurse –
(a) Peeth karke (b) Arm's length se door, angle par khadi, exit clear (c) Akeli jaaye (d) Bilkul paas face-to-face

20. Patient restrain karna pade to –
(a) Akela nurse (b) Punishment ke liye (c) Bina order ke din bhar (d) Trained team (≥ 4–5), last resort, document, 15-min checks

21. Independent (voluntary) patient discharge maangta hai. MHCA –
(a) Kabhi nahi (b) 1 mahina rokna (c) Court order (d) Discharge request maani jaati hai (assessment ke baad, 24 hrs ke andar)

22. Community mental health nurse ka role NAHI –
(a) Depot injection follow-up (b) Psychoeducation (c) Chaining patient at home (d) Home visit

23. Schizophrenia patient discharge ho raha hai. Relapse rokne ke liye –
(a) Kuch nahi (b) Isolation (c) Family psychoeducation, compliance, early warning signs (d) Dawa band

24. Wandering mentally ill person police ko mile – MHCA –
(a) Ghar band (b) Chhod dena (c) Jail (d) Public mental health establishment le jaana

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. Sec 5 Q. Sec 94 R. Sec 95 S. Sec 115 — with 1. Attempted suicide 2. Prohibited procedures 3. Emergency treatment 72 hrs 4. Advance directive
(a) P-4, Q-3, R-2, S-1 (b) P-3, Q-4, R-2, S-1 (c) P-4, Q-2, R-3, S-1 (d) P-1, Q-3, R-2, S-4

26. De-escalation technique NAHI –
(a) Exit clear (b) Calm soft voice (c) Patient ko challenge karna (d) Choices dena

27. Assertion (A): MHCA 2017 rights-based act hai. Reason (R): Isme advance directive, nominated representative aur insurance parity jaise rights diye gaye.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Tertiary prevention in mental health –
(a) Life skills education (b) Early screening (c) Rehabilitation, relapse prevention (d) Genetic counselling

29. Assault cycle ka sahi order –
(a) Trigger → escalation → crisis → recovery → post-crisis depression (b) Crisis → trigger (c) Random (d) Recovery → crisis

30. MHCA ke anusar mental illness ki definition me include NAHI –
(a) Mood disorder (b) Mental retardation / intellectual disability (c) Disorder of thinking (d) Perception disorder

✓ Answer Key & Explanation

1-b Past violence.
2-c De-escalation.
3-a 1982.
4-d Bellary.
5-d 29 May 2018.
6-a Sec 5.
7-d 72 hrs.
8-c 30 days.

9-a Sec 95.
10-c MHRB.
11-d 14416.
12-d Transitional.
13-d McNaughten.
14-a Testamentary.
15-c Tarasoff.
16-c Lorazepam/ ECT.
17-c $NaHCO_3$.
18-a Prohibited.
19-b Safe distance.
20-d Team, last resort.
21-d Discharge.
22-c Chaining prohibited.
23-c Psychoeducation.
24-d Sec 100.
25-a Sahi.
26-c Challenging escalates.
27-a Dono sahi, R reason hai.
28-c Rehabilitation.
29-a Assault cycle.
30-b ID excluded.

12 GSSSB Pattern Mock Test – 100 MCQs (Full Mental Health Syllabus)



Mock test ke rules

Score guide: 80+ Excellent · 65–79 Good · 50–64 Weak topics revise karo · < 50 Notes dobara padho.

? Mock Test – Part A (1–50)

1. Removed chains –
(a) Beck (b) Freud (c) Pinel (d) Bleuler

2. Schizophrenia term –
(a) Schneider (b) Kraepelin (c) Bleuler (d) Jung

3. Mother of psychiatric nursing –
(a) Peplau (b) Dix (c) Richards (d) Nightingale

4. ECT introduced –
(a) Sakel (b) Moniz (c) Meduna (d) Cerletti & Bini

5. ICD –
(a) UNICEF (b) APA (c) WHO (d) ICN

6. Hallucination –
(a) Perception without stimulus (b) Intrusive thought (c) False belief (d) Misinterpretation

7. Illusion –
(a) No stimulus (b) Memory gap (c) Misinterpretation of real stimulus (d) Belief

8. Delusion –
(a) Fixed false belief (b) Habit (c) Mood (d) Perception

9. Commonest delusion –
(a) Jealousy (b) Grandeur (c) Nihilism (d) Persecution

10. Flight of ideas –
(a) Depression (b) Mania (c) OCD (d) Schizophrenia

11. Confabulation –
(a) Mania (b) Phobia (c) Korsakoff (d) PTSD

12. Waxy flexibility –
(a) GAD (b) Catatonia (c) Delirium (d) Mania

13. Conscious defence –
(a) Suppression (b) Denial (c) Repression (d) Projection

14. Mature defence –
(a) Projection (b) Denial (c) Sublimation (d) Regression

15. Orientation lost first –
(a) Time (b) Person (c) Self (d) Place

16. MMSE max –
(a) 20 (b) 30 (c) 50 (d) 100

17. Abstract thinking test –
(a) Serial 7s (b) Proverbs (c) Digit span (d) Orientation

18. Transference –
(a) Empathy (b) Nurse's onto patient (c) Sympathy (d) Patient's feelings onto nurse

19. Non-therapeutic –
(a) Clarification (b) Silence (c) Reflection (d) False reassurance

20. Trust & contract phase –
(a) Orientation (b) Working (c) Identification (d) Termination

21. First antipsychotic –
(a) Clozapine (b) Chlorpromazine (c) Haloperidol (d) Risperidone

22. Agranulocytosis –
(a) Clozapine (b) Aripiprazole (c) Olanzapine (d) Haloperidol

23. Irreversible EPS –
(a) Parkinsonism (b) Dystonia (c) Tardive dyskinesia (d) Akathisia

24. Acute dystonia Rx –
(a) Propranolol (b) Fluoxetine (c) IM promethazine (d) Lithium

25. NMS drug –
(a) Diazepam (b) Dantrolene (c) Haloperidol (d) Naloxone

26. First-line antidepressant –
(a) TCA (b) SSRI (c) BZD (d) MAOI

27. Lethal in overdose –
(a) Mirtazapine (b) Buspirone (c) SSRI (d) TCA

28. Cheese reaction –
(a) Lithium (b) Clozapine (c) MAOI (d) SSRI

29. Lithium level –
(a) 0.1–0.3 (b) 0.6–1.2 mEq/L (c) 2–3 (d) 5

30. Lithium teratogen –
(a) Ebstein's anomaly (b) Phocomelia (c) Cleft (d) NTD

31. BZD antidote –
(a) Naloxone (b) Atropine (c) Flumazenil (d) Dantrolene

32. Non-addictive anxiolytic –
(a) Lorazepam (b) Buspirone (c) Diazepam (d) Alprazolam

33. ECT main indication –
(a) PD (b) Suicidal depression (c) Insomnia (d) Anxiety

34. ECT relative contraindication –
(a) Raised ICP (b) Pregnancy (c) Diabetes (d) Age

35. ECT NPO –
(a) 6–8 hrs (b) 1 hr (c) 24 hrs (d) None

36. ECT muscle relaxant –
(a) Thiopentone (b) Diazepam (c) Atropine (d) Succinylcholine

37. ECT common side effect –
(a) Fracture (b) Blindness (c) Memory loss (d) Death

38. CBT –
(a) Wolpe (b) Beck (c) Freud (d) Moreno

39. Phobia therapy –
(a) Psychoanalysis (b) ECT (c) Lithium (d) Systematic desensitisation

40. OCD therapy –
(a) ECT (b) Hypnosis (c) ERP (d) Aversion

41. Therapeutic community –
(a) Rogers (b) Peplau (c) Caplan (d) Maxwell Jones

42. Crisis duration –
(a) 1 year (b) 6 months (c) 1 day (d) 4–6 weeks

43. Schizophrenia prevalence –
(a) 0.01 % (b) 1 % (c) 10 % (d) 25 %

44. FRS –
(a) Bleuler (b) Beck (c) Kurt Schneider (d) Freud

45. Commonest schizophrenia type –
(a) Catatonic (b) Paranoid (c) Simple (d) Hebephrenic

46. Worst prognosis –
(a) Hebephrenic (b) Catatonic (c) Undifferentiated (d) Paranoid

47. Negative symptom –
(a) Hallucination (b) Delusion (c) Thought broadcasting (d) Avolition

48. Resistant schizophrenia –
(a) Haloperidol (b) SSRI (c) Clozapine (d) Lithium

49. Hallucinating patient –
(a) Agree (b) Argue (c) Present reality (d) Ignore

50. Shared psychosis –
(a) Ganser (b) Folie à deux (c) Cotard (d) Capgras

? Mock Test – Part B (51–100)

51. Depression duration –
(a) 2 weeks (b) 6 months (c) 1 week (d) 1 month

52. Depression sleep –
(a) Night terror (b) Sleepwalking (c) Hypersomnia (d) Early morning awakening

53. Mania duration –
(a) 1 week (b) 4 days (c) 1 day (d) 2 weeks

54. Bipolar II –
(a) Mania (b) Hypomania + depression (c) Cyclothymia (d) Dysthymia

55. Bipolar prophylaxis –
(a) Fluoxetine (b) Lithium (c) Diazepam (d) Haloperidol

56. Manic activity –
(a) Chess (b) Cards (c) Football match (d) Walking/ sweeping

57. Suicide predictor –
(a) Married (b) Previous attempt (c) Female (d) Rich

58. Suicide method India –
(a) Firearm (b) Hanging (c) Jumping (d) Drowning

59. Suicide scale –
(a) MMSE (b) SAD PERSONS (c) AIMS (d) CAGE

60. Suicide risk ↑ when –
(a) Deep stupor (b) Depression lifts (c) Eating (d) Asleep

61. Sec 115 –
(a) ECT (b) Advance directive (c) Emergency (d) Suicide attempt

62. GAD –
(a) 1 week (b) 1 month (c) 6 months (d) 2 weeks

63. Acrophobia –
(a) Water (b) Heights (c) Closed space (d) Dark

64. Claustrophobia –
(a) Strangers (b) Closed spaces (c) Crowds (d) Heights

65. Commonest compulsion –
(a) Hoarding (b) Praying (c) Washing (d) Counting

66. PTSD –
(a) < 1 week (b) > 1 month (c) 2 days (d) 1 year

67. GAS –
(a) Selye (b) Rogers (c) Beck (d) Freud

68. Grief stages –
(a) Peplau (b) Maslow (c) Erikson (d) Kübler-Ross

69. La belle indifférence –
(a) Mania (b) Conversion (c) OCD (d) Depression

70. Malingering –
(a) External gain (b) Sick role (c) Unconscious (d) Psychosis

71. Tolerance –
(a) Relapse (b) Withdrawal (c) Need more dose (d) Craving

72. CAGE –
(a) Depression (b) Anxiety (c) Alcohol (d) Dementia

73. Earliest withdrawal sign –
(a) DT (b) Tremors (c) Seizure (d) Hallucinosis

74. DT onset –
(a) 2 weeks (b) 6 hrs (c) 12 hrs (d) 48–72 hrs

75. Detox drug –
(a) Haloperidol (b) Naloxone (c) Chlordiazepoxide (d) Disulfiram

76. Thiamine before –
(a) Insulin (b) Glucose (c) Saline (d) Food

77. Wernicke deficiency –
(a) Niacin (b) Folate (c) Thiamine (d) B12

78. Disulfiram inhibits –
(a) COMT (b) Alcohol dehydrogenase (c) Aldehyde dehydrogenase (d) MAO

79. Opioid antidote –
(a) Naloxone (b) Flumazenil (c) Atropine (d) Dantrolene

80. Opioid pupils –
(a) Unequal (b) Dilated (c) Normal (d) Pin-point

81. NDPS Act –
(a) 1949 (b) 1971 (c) 1985 (d) 2017

82. Antisocial PD –
(a) Perfectionism (b) No guilt (c) Dependency (d) Shyness

83. Borderline PD therapy –
(a) DBT (b) ECT (c) Lithium only (d) Surgery

84. Highest mortality –
(a) Anorexia nervosa (b) GAD (c) OCD (d) Phobia

85. Russell's sign –
(a) Bulimia (b) Pica (c) Obesity (d) Anorexia

86. Commonest sleep disorder –
(a) Apnoea (b) Insomnia (c) Bruxism (d) Narcolepsy

87. Cataplexy –
(a) Night terror (b) RLS (c) Insomnia (d) Narcolepsy

88. Delirium –
(a) Insidious, clear (b) Chronic stable (c) Irreversible (d) Acute, fluctuating, attention ↓

89. Commonest dementia –
(a) Lewy body (b) Alzheimer's (c) Vascular (d) Pick's

90. Alzheimer's drug –
(a) Donepezil (b) Haloperidol (c) Methylphenidate (d) Lithium

91. Autism onset –
(a) < 12 yrs (b) Adult (c) < 18 yrs (d) < 3 yrs

92. ADHD drug –
(a) Diazepam (b) Haloperidol (c) Methylphenidate (d) Lithium

93. Mild ID IQ –
(a) 50–69 (b) < 20 (c) 20–34 (d) 35–49

94. Violence predictor –
(a) Past violence (b) Female (c) Education (d) Age

95. First step with aggression –
(a) Restraint (b) Injection (c) De-escalation (d) Seclusion

96. NMHP –
(a) 1982 (b) 1987 (c) 1996 (d) 2017

97. MHCA in force –
(a) 29 May 2018 (b) 1987 (c) 2014 (d) 2017 Jan

98. Emergency treatment MHCA –
(a) 7 days (b) 24 hrs (c) 30 days (d) 72 hrs

99. Tele-MANAS –
(a) 108 (b) 181 (c) 1098 (d) 14416

100. World Mental Health Day –

(a) 7 April (b) 10 September (c) 10 October (d) 26 June

Answer Key – Mock Test (with explanation)

1-c Pinel.
2-c Bleuler.
3-a Peplau.
4-d Cerletti & Bini.
5-c WHO.
6-a Hallucination.
7-c Illusion.
8-a Delusion.
9-d Persecution.
10-b Mania.
11-c Korsakoff.
12-b Catatonia.
13-a Suppression.
14-c Sublimation.
15-a Time.
16-b 30.
17-b Proverbs.
18-d Transference.
19-d False reassurance.
20-a Orientation.
21-b Chlorpromazine.
22-a Clozapine.
23-c TD.
24-c Promethazine.
25-b Dantrolene.
26-b SSRI.
27-d TCA.
28-c MAOI.
29-b 0.6–1.2.
30-a Ebstein's.
31-c Flumazenil.
32-b Buspirone.
33-b Suicidal depression.
34-a Raised ICP.
35-a 6–8 hrs.
36-d Succinylcholine.
37-c Memory loss.
38-b Beck.
39-d Desensitisation.
40-c ERP.
41-d Maxwell Jones.
42-d 4–6 weeks.
43-b 1 %.
44-c Schneider.
45-b Paranoid.
46-a Hebephrenic.
47-d Avolition.
48-c Clozapine.
49-c Present reality.
50-b Folie à deux.
51-a 2 weeks.
52-d Early morning awakening.
53-a 1 week.
54-b Hypomania + depression.
55-b Lithium.
56-d Walking/ sweeping.
57-b Previous attempt.
58-b Hanging.
59-b SAD PERSONS.
60-b Depression lifts.
61-d Suicide attempt.
62-c 6 months.
63-b Heights.
64-b Closed spaces.
65-c Washing.
66-b > 1 month.
67-a Selye.
68-d Kübler-Ross.
69-b Conversion.
70-a External gain.
71-c Tolerance.
72-c Alcohol.
73-b Tremors.
74-d 48–72 hrs.
75-c Chlordiazepoxide.
76-b Glucose.
77-c Thiamine.
78-c ALDH.
79-a Naloxone.
80-d Pin-point.
81-c 1985.
82-b No guilt.
83-a DBT.
84-a Anorexia.
85-a Bulimia.
86-b Insomnia.
87-d Narcolepsy.
88-d Delirium.
89-b Alzheimer's.
90-a Donepezil.
91-d < 3 yrs.
92-c Methylphenidate.
93-a 50–69.
94-a Past violence.
95-c De-escalation.
96-a 1982.
97-a 29 May 2018.
98-d 72 hrs.

99-d 14416.
100-c 10 October.

— Mental Health Nursing · GSSSB Staff Nurse Preparation · Padhte raho, revise karte raho! —